This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

Mounjaro (tirzepatide) Injection

for weight management

Patient Group Direction, version 011, issued 30 September 2026. Supersedes Version 010, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE guidance and the SmPC for Mounjaro (tirzepatide) injection for weight management in adults

NICE technology appraisal guidance TA1026, Tirzepatide for managing overweight and obesity, published 23 December 2024, last updated 1 September 2025 (overview and recommendations chapter read, including the What this means in practice summary of NHS England's interim commissioning guidance). Mounjaro KwikPen 2.5 mg, 5 mg, 7.5 mg, 10 mg, 12.5 mg and 15 mg solution for injection in pre-filled pen, Summary of Product Characteristics, Eli Lilly and Company Limited, last updated on emc 8 September 2026 (medicines.org.uk product 15481), sections 1 to 4.8 read and summarised 10 September 2026; section 6 (text revised 29 August 2026) read and summarised 11 September 2026.

NICE eligibility (TA1026 recommendation 1.1)

Tirzepatide is recommended as an option for managing overweight and obesity, alongside a reduced-calorie diet and increased physical activity in adults, only if they have an initial BMI of at least 35 kg/m2, and at least 1 weight-related comorbidity, and the company provides it according to the commercial arrangement.

Use a lower BMI threshold (usually reduced by 2.5 kg/m2) for people from South Asian, Chinese, other Asian, Middle Eastern, Black African or African-Caribbean ethnic backgrounds.

NICE did not recommend tirzepatide for the wider group proposed by the company (BMI of at least 30 kg/m2 with at least 1 weight-related comorbidity) because the cost-effectiveness estimates for that group were above the range NICE considers acceptable.

NICE notes that tirzepatide can be used in primary care or specialist weight management services, and that indirect comparisons suggest it is more effective than semaglutide alongside diet and exercise support.

NICE stopping rule (TA1026 recommendations 1.2 and 1.3)

If less than 5 percent of the initial weight has been lost after 6 months on the highest tolerated dose, decide whether to continue treatment, taking into account the benefits and risks of treatment for the person.

The recommendations are not intended to affect treatment started in the NHS before publication; people having treatment outside the recommendations may continue under their existing funding arrangements until they and their NHS healthcare professional consider it appropriate to stop.

NICE phased rollout (TA1026, What this means in practice)

Tirzepatide must be funded in the NHS in England for adults with overweight and obesity if it is the most suitable option, but a funding variation means it is being introduced in phases over 3 years from publication, as set out in NHS England's interim commissioning guidance.

Eligibility in the phased rollout depends on BMI and on how many of these weight-related conditions the person has: high blood pressure, dyslipidaemia, obstructive sleep apnoea, cardiovascular disease, and type 2 diabetes.

Eligible from publication: 4 or more of the listed conditions and a BMI of 40 kg/m2 or more.

From around June 2026: 4 or more of the listed conditions and a BMI between 35 kg/m2 and 39.9 kg/m2.

From around March 2027: 3 of the listed conditions and a BMI of 40 kg/m2 or more.

For people from South Asian, Chinese, other Asian, Middle Eastern, Black African or African-Caribbean ethnic backgrounds the qualifying BMIs are 2.5 kg/m2 lower (for example 37.5 kg/m2 instead of 40 kg/m2).

NICE has published local formulary information, a guide for prescribing medicines to manage overweight and obesity, and downloadable checklists for initial assessment, counselling and follow-up of people having tirzepatide.

Licensed indication (SmPC section 4.1)

For weight management, Mounjaro is indicated, including weight loss and weight maintenance, as an adjunct to a reduced-calorie diet and increased physical activity in adults with an initial BMI of 30 kg/m2 or more (obesity), or 27 kg/m2 to less than 30 kg/m2 (overweight) in the presence of at least one weight-related comorbid condition (for example hypertension, dyslipidaemia, obstructive sleep apnoea, cardiovascular disease, prediabetes, or type 2 diabetes mellitus).

Mounjaro is also indicated for insufficiently controlled type 2 diabetes in adults, adolescents and children aged 10 years and above; that indication is outside the scope of this summary.

The safety and efficacy of tirzepatide for weight management have not been established in children and adolescents aged less than 18 years.

Dose escalation (SmPC section 4.2)

The starting dose is 2.5 mg once weekly. After 4 weeks, the dose should be increased to 5 mg once weekly.

If needed, dose increases can be made in 2.5 mg increments after a minimum of 4 weeks on the current dose.

The recommended maintenance doses in adults are 5 mg, 10 mg and 15 mg. The maximum dose is 15 mg once weekly.

For weight management, if patients have been unable to lose at least 5 percent of their initial body weight 6 months after titrating to the highest tolerated dose, a decision is required on whether to continue treatment, taking into account the benefit and risk profile in the individual patient.

Missed dose: administer as soon as possible within 4 days after the missed dose; if more than 4 days have passed, skip the missed dose and give the next dose on the regularly scheduled day.
The day of weekly administration can be changed if necessary as long as the time between two doses is at least 3 days.
When tirzepatide is added to a sulphonylurea and/or insulin, a dose reduction of the sulphonylurea or insulin may be considered to reduce hypoglycaemia risk; blood glucose self-monitoring is necessary and a stepwise approach to insulin reduction is recommended. Existing metformin and/or SGLT2 inhibitor doses can be continued.

Special populations (SmPC section 4.2)

No dose adjustment is needed based on age, gender, race, ethnicity or body weight.
No dose adjustment is required for renal impairment including end-stage renal disease, but experience in severe renal impairment and ESRD is limited and caution should be exercised.
No dose adjustment is required for hepatic impairment; experience in severe hepatic impairment is limited and caution should be exercised.
Only very limited data are available from patients aged 85 years and over.

Presentation, method of administration and storage (SmPC sections 2, 4.2 and 6)

The SmPC covers the Mounjaro KwikPen multiple-dose pre-filled pen in 2.5 mg, 5 mg, 7.5 mg, 10 mg, 12.5 mg and 15 mg strengths; each dose is 0.6 mL and each pen contains 4 doses.
Each 0.6 mL dose contains 5.4 mg benzyl alcohol (E1519), which may cause allergic reactions; patients with hepatic or renal impairment should be told of the potential risk of metabolic acidosis from accumulation of benzyl alcohol over time.
Mounjaro is injected subcutaneously in the abdomen or thigh, or another person may inject into the back of the upper arm. It can be given at any time of day, with or without meals.
Injection sites should be rotated with each dose, and if the patient also injects insulin, Mounjaro should be injected at a different site.
Patients and caregivers should be advised to read the instructions for use and package leaflet for the KwikPen carefully before administering the product.
The emc search for Mounjaro on 10 September 2026 returned only the six KwikPen products; no vial presentation is listed on emc.
Storage, shelf life and in-use period (SmPC sections 6.3, 6.4 and 6.6): store in a refrigerator (2°C to 8°C) and do not freeze; Mounjaro that has been frozen must not be used.
The shelf life before first use is 24 months. After first use, the KwikPen may be stored unrefrigerated for up to 30 days at a temperature not above 30°C and must then be discarded.
Each KwikPen contains 4 doses of 0.6 mL; any excess solution in the pen after use should be discarded. Inspect the product visually before use and discard it for particulate matter or discolouration. Needles are not included in the pack; packs contain 1 or 3 pre-filled KwikPens (section 6.5).

Contraindications (SmPC section 4.3)

Hypersensitivity to the active substance or to any of the excipients listed in section 6.1. No other contraindications are listed.

Warnings and precautions (SmPC section 4.4)

Acute pancreatitis: not studied in patients with a history of pancreatitis, use with caution. Acute pancreatitis, including post-marketing necrotising pancreatitis and fatal cases, has been reported with tirzepatide. Inform patients of the symptoms (persistent, severe abdominal pain) and to seek immediate medical attention; discontinue if suspected and do not restart if confirmed. Raised pancreatic enzymes alone are not predictive.
Hypoglycaemia in type 2 diabetes: patients on tirzepatide with an insulin secretagogue (for example a sulphonylurea) or insulin may have an increased risk, which may be lowered by reducing the secretagogue or insulin dose.
Gastrointestinal effects: nausea, vomiting and diarrhoea may lead to dehydration, which could cause deterioration in renal function including acute renal failure; advise patients of the risk and of precautions to avoid fluid depletion and electrolyte disturbances, particularly in the elderly.
Severe gastrointestinal disease: not studied in severe gastrointestinal disease including severe gastroparesis; use with caution.
Diabetic retinopathy: rapid improvement in glucose control has been associated with temporary worsening of diabetic retinopathy; patients with diabetic retinopathy should be monitored closely and treated according to clinical guidelines.
Aspiration with general anaesthesia or deep sedation: pulmonary aspiration has been reported in patients on GLP-1 receptor agonists; the increased risk of residual gastric content due to delayed gastric emptying should be considered before such procedures.

The product is essentially sodium-free.

Interactions, including oral contraceptives (SmPC section 4.5)

Tirzepatide delays gastric emptying and can reduce the rate of absorption of concomitant oral medicines (decreased Cmax and delayed tmax), most pronounced at treatment initiation.

No dose adjustments are expected for most oral medicines, but patients on oral medicines with a narrow therapeutic index (for example warfarin, digoxin) should be monitored, especially at initiation and after dose increases; the risk of delayed effect should also be considered for oral medicines where rapid onset matters.

Paracetamol: no dose adjustment needed; after a single 5 mg dose of tirzepatide paracetamol Cmax fell by 50 percent with tmax delayed by 1 hour, with no effect on overall exposure and no effect after four weekly doses.

Oral contraceptives: a single 5 mg dose of tirzepatide reduced the Cmax of ethinyl estradiol by 59 percent and AUC by 20 percent, and norelgestromin Cmax by 55 percent and AUC by 23 percent, with tmax delayed by around 4 hours; this is not considered clinically relevant and no dose adjustment is required in women with normal BMI.

Because there is limited information in women with obesity or overweight and reduced efficacy of oral contraceptives cannot be excluded, the SmPC advises switching to a non-oral contraceptive method, or adding a barrier method, for 4 weeks after initiating tirzepatide and for 4 weeks after each dose escalation.

Pregnancy, breastfeeding and contraception (SmPC section 4.6)

Tirzepatide should not be used during pregnancy and is not recommended in women of childbearing potential who are not using contraception; animal studies have shown reproductive toxicity.

If a patient wishes to become pregnant, tirzepatide should be discontinued at least 1 month before a planned pregnancy because of its long half-life.

Breast-feeding: in a study of 11 women tirzepatide in breast milk was undetectable to very low; any low amount is expected to be degraded and not absorbed intact by the infant, and the SmPC states that tirzepatide could be considered for use during breast-feeding, while noting it is not known whether reduced maternal food intake affects milk composition.

The effect on human fertility is unknown; animal studies did not indicate direct harmful effects.

Driving (SmPC section 4.7)

No or negligible influence on the ability to drive or use machines; patients on a sulphonylurea or insulin should take precautions to avoid hypoglycaemia while driving.

Adverse effects (SmPC section 4.8)

The most frequently reported adverse reactions are gastrointestinal, mostly mild or moderate, occurring primarily during dose escalation and decreasing over time.

Very common (1 in 10 or more): hypoglycaemia when used with a sulphonylurea or insulin (type 2 diabetes trials), decreased appetite, nausea, diarrhoea, vomiting, constipation, abdominal pain, fatigue (vomiting, constipation, abdominal pain and fatigue are very common in the weight management trials).

Common (1 in 100 to 1 in 10): hypersensitivity reactions, hypoglycaemia with metformin and SGLT2 inhibitor, dizziness, hypotension-related events, dyspepsia, abdominal distension, eructation, flatulence, gastro-oesophageal reflux disease, cholelithiasis, cholecystitis, hair loss, injection site reactions, heart rate increased, blood calcitonin increased, lipase increased, amylase increased.

Uncommon (1 in 1,000 to 1 in 100): hypoglycaemia with metformin alone, weight decreased, dysgeusia, dysaesthesia, acute pancreatitis, delayed gastric emptying, injection site pain. Rare: anaphylactic reaction and angioedema (post-marketing).

In the pooled SURMOUNT-1 and SURMOUNT-2 weight management trials, gastrointestinal disorders occurred in about 56 percent of patients on tirzepatide 5 mg, 10 mg or 15 mg versus 29.7 percent on placebo; nausea in 24.6 percent, 29.0 percent and 28.0 percent (8.5 percent placebo) and diarrhoea in 18.7 percent, 20.8 percent and 22.5 percent (7.8 percent placebo). Permanent discontinuation due to gastrointestinal events was 1.9 percent, 3.3 percent and 4.3 percent by dose versus 0.5 percent on placebo.

Hypersensitivity reactions were reported in 5.0 percent of tirzepatide-treated patients in the pooled weight management trials (3.8 percent placebo), sometimes severe (for example urticaria, eczema, dermatitis and rash).

In the weight management trials acute gallbladder disease was reported in 2.0 percent of tirzepatide-treated patients (1.6 percent placebo), cholecystitis in 0.6 percent (0.2 percent placebo) and cholelithiasis in 1.1 percent (1.0 percent placebo); acute gallbladder events were positively correlated with weight reduction.

Hair loss was reported in 4.9 percent of tirzepatide-treated patients in the weight management trials (1.0 percent placebo), mainly mild, with most patients recovering on continued treatment.

In SURMOUNT-2 (patients with type 2 diabetes), clinically significant hypoglycaemia occurred in 4.2 percent of tirzepatide-treated patients versus 1.3 percent on placebo, with risk increased when used with a sulfonylurea; no severe hypoglycaemia was reported.

In type 2 diabetes trials tirzepatide produced a maximum mean increase in heart rate of 3 to 5 beats per minute.

Mounjaro is subject to additional monitoring (black triangle); suspected adverse reactions should be reported via the MHRA Yellow Card scheme.

What the sources do not say

TA1026 does not address community pharmacy or private supply; it sets the NHS England eligibility and phased rollout described above.

The Mounjaro SmPC does not mention suicidal ideation or behaviour, does not carry a NAION warning, and has no separate gallbladder disease warning in section 4.4 (gallbladder events are described in section 4.8).

The SmPC on emc does not describe a vial presentation; only KwikPen products are listed.

SmPC section 6 gives no instruction about keeping the pen in the carton or protecting it from light, and says nothing about travel.

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must be competent in weight management consultation, BMI calculation and the recognition of red flags requiring referral • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005

Initial Assessment

Before commencing treatment with weight management therapies, the patient must be assessed by a registered healthcare professional. The initial assessment should be face to face. The assessment should include, but may not be limited to:

- Causes of weight gain. Refer the patient to their GP if prescribed medication is causing weight gain.
- The patient’s lifestyle, diet and level of exercise.
- Has the patient tried to lose weight in the past? Discuss what has and has not worked.
- Other factors that may be causing weight gain, including mental health, environmental and psychological factors. Consider signposting to another health professional, for example a mental health team.
- Other disease states that may predispose to weight gain. Consider referring to the GP for further advice.

- The patient's expectations of weight loss, and whether they are realistic for their height and body frame.
- Calculate BMI and determine the ideal weight. Set a realistic target weight and agree review intervals.

Clinical condition or situation to which this PGD applies

PGD Indication	Mounjaro (tirzepatide) is a dual GIP/GLP-1 receptor agonist indicated as an adjunct to a reduced-calorie diet and increased physical activity for weight management, including weight loss and weight maintenance, in adults with an initial Body Mass Index (BMI) of 30 kg/m² or above (obesity), or 27 kg/m² to below 30 kg/m² (overweight) in the presence of at least one weight-related comorbidity, for example dysglycaemia (pre-diabetes or type 2 diabetes mellitus), hypertension, dyslipidaemia, obstructive sleep apnoea or cardiovascular disease.
Inclusion criteria	• Adults aged 18 to 75 years (inclusive). • BMI 30 kg/m² or above, or BMI 27 kg/m² or above with at least one weight-related comorbidity (for example hypertension, type 2 diabetes mellitus, dyslipidaemia, obstructive sleep apnoea, established cardiovascular disease). • Patient is willing to follow a reduced-calorie diet and increase physical activity in line with the agreed lifestyle plan. • Initial assessment completed and documented (see the Initial Assessment section above). • Informed consent obtained, including off-label considerations where applicable, the side-effect profile and treatment expectations. • No exclusion criteria present.
Exclusion criteria	• Patients under 18 years of age. • Adults aged over 75 years (upper age limit under this PGD; refer to a specialist if treatment is being considered). • History of suicidal ideation or active severe mental illness where psychiatric oversight is absent and there is current concern about mood or risk (see cautions). • Pregnancy, breastfeeding, or planning pregnancy. Effective contraception is required; advise discontinuation at least 1 month before a planned conception (SmPC). • Known hypersensitivity to tirzepatide or to any of the excipients. • Personal or family history of medullary thyroid carcinoma (MTC) or Multiple Endocrine Neoplasia syndrome type 2 (MEN 2). This exclusion, like pancreatitis, is stricter than the UK SmPC, which lists no contraindication other than hypersensitivity; it follows the US labelling and is a deliberate service restriction. • History of pancreatitis, acute or chronic. • Severe gastrointestinal disease, including gastroparesis or severe persistent gastrointestinal disorder. • Current cholelithiasis (gallstones) or cholecystitis, or cholecystectomy within the last 3 months. • Concurrent use of any other GLP-1 receptor agonist or insulin secretagogue, FOR ANY INDICATION. Ask specifically about medicines taken for diabetes and name the products: oral or injectable semaglutide, tirzepatide, orforglipron, liraglutide, dulaglutide, exenatide, and the sulfonylureas and meglitinides. Patients do not always think of a diabetes medicine as the same

	kind of drug as a weight loss one. ● Type 1 diabetes mellitus. ● Diabetic retinopathy. Treatment may worsen retinopathy; defer or refer to a specialist. ● Insulin-treated diabetes. Refer: a pharmacy weight-management service cannot manage insulin dose reduction. ● Severe renal impairment (eGFR below 30 mL/min/1.73 m²) or end-stage renal disease. ● Severe hepatic impairment. ● Known diagnosis of heart failure with reduced ejection fraction below 40%. ● Active eating disorder (anorexia nervosa, bulimia, or binge-eating disorder under specialist care). ● BMI below the PGD inclusion threshold. ● Patients who, in the clinical judgement of the healthcare professional, are not suitable for the medication.
Cautions	● Counsel on gradual dose escalation and gastrointestinal side effects; consider delaying titration or reducing to the previous dose if significant symptoms occur. ● History of suicidal ideation, or active severe mental illness: supply only where appropriate psychiatric oversight is in place; monitor mood at review and refer if there is any concern. Where oversight is absent and concern exists, the exclusion criteria apply. ● Mild to moderate renal impairment: monitor for dehydration secondary to gastrointestinal side effects. ● An endocrine cause of obesity is not an exclusion under this PGD: a patient with treated hypothyroidism or treated Cushing's syndrome may be supplied. Where an endocrine cause is suspected and has not been assessed or treated, inform the GP so that it can be investigated and record this in the notes; this is a caution to inform the GP, not a reason to withhold supply. ● Concomitant oral medications: GLP-1 and GIP agents delay gastric emptying. Counsel patients on possible reduced absorption of oral medications, especially narrow therapeutic index drugs and oral contraceptives; non-oral contraception or a barrier method is advised during titration and for 4 weeks after each dose increase. ● For individuals who take HRT, due to the lack of data regarding absorption, non-oral products (for example patch, gel, or levonorgestrel intrauterine device) may be considered. ● Tirzepatide delays gastric emptying and thereby has the potential to impact the rate of absorption of concomitantly administered oral medicinal products, especially those with a narrow therapeutic index. Upon initiation of tirzepatide treatment in patients on warfarin, frequent monitoring of INR is recommended. ● Cases of pulmonary aspiration have been reported in patients receiving GLP-1 receptor agonists undergoing general anaesthesia or deep sedation for surgery. The increased risk of residual gastric content due to delayed gastric emptying should therefore be considered before procedures under general anaesthesia or deep sedation.

	• Cases of tachycardia have been reported. In patients with a pre-existing increased heart rate, use with caution and seek specialist advice before use. If a patient experiences a clinically relevant sustained increase in resting heart rate, treatment should be discontinued and advice sought. • Counsel on the signs and symptoms of pancreatitis; discontinue and refer urgently if suspected. • Type 2 diabetes on metformin, an SGLT2 inhibitor or a DPP-4 inhibitor only: no dose adjustment is needed, but inform the GP. Any sulfonylurea, meglitinide or insulin EXCLUDES (see exclusion criteria); there is no GP-monitored route for those patients under this PGD. • Risk of dehydration. Counsel on adequate fluid intake during gastrointestinal side effects. • If less than 5% of initial body weight has been lost after 6 months on the maximum tolerated dose, a decision on whether to continue is required and recorded (see monitoring and review). • Rotate injection sites to reduce local irritation.
Actions if patient is excluded or declines treatment	• Discuss the reason for exclusion with the patient and ensure they understand. • Advise on alternative treatment options and how these can be accessed, for example the GP, a specialist weight management service, or lifestyle programmes. • If the exclusion relates to an undiagnosed or unmanaged comorbidity, recommend GP review. • Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Mounjaro (tirzepatide) solution for injection in a multi-dose pre-filled pen (KwikPen). Available strengths: 2.5 mg, 5 mg, 7.5 mg, 10 mg, 12.5 mg and 15 mg per 0.6 mL dose; each pen contains 4 doses (2.4 mL).
Legal category	POM
Storage	• Store in a refrigerator (2°C to 8°C). Do not freeze; discard if frozen. • After first use the KwikPen may be stored unrefrigerated at a temperature not above 30°C for up to 30 days, and must then be discarded (SmPC section 6.4). Before first use keep the pen in the refrigerator and do not use it after the expiry date on the label (shelf life 24 months). • Inspect the solution before each use and do not use the pen if the solution contains particles or is discoloured. Any solution left in the pen after the fourth dose is discarded (SmPC section 6.6). • Travel: carry pens in hand luggage when travelling by air. Pens should not be placed in checked baggage where temperatures cannot be guaranteed. A travel letter from the prescriber or pharmacist may be required.
Route/method of administration	• Subcutaneous injection in the abdomen, thigh or upper arm. Rotate injection sites. • Administered once weekly, on the same day each week, with or without food, at any time of day. • The day of weekly administration can be changed if necessary as long as the time between two doses is at least 3 days (more than

	72 hours). After selecting a new dosing day, once-weekly dosing should be continued. ● Do not administer intravenously or intramuscularly.
Dose and frequency	● Starting dose: 2.5 mg once weekly for 4 weeks. This is a titration dose and is not intended for sustained weight management. ● After 4 weeks at 2.5 mg, increase to 5 mg once weekly. ● Further increases of 2.5 mg may be made after a minimum of 4 weeks on the current dose if additional weight management is required and the current dose is tolerated. ● Recommended maintenance doses: 5 mg, 10 mg or 15 mg once weekly. Maximum dose 15 mg once weekly. ● If a dose is missed, administer as soon as possible within 4 days (96 hours). If more than 4 days have passed, skip the missed dose and resume the usual schedule. Do not administer two doses within 3 days. ● If significant gastrointestinal symptoms occur, consider delaying dose escalation or reducing to the previous dose until symptoms have improved. ● If more than 2 doses are missed, reduce and re-escalate the dose to avoid unwanted side effects when treatment is re-initiated.
Quantity to be administered and/or supplied	● One KwikPen contains four weekly doses (4 x 0.6 mL) at the prescribed strength. ● Supply ONE pen (4 weeks of treatment) per patient appointment. ● If the patient is going on holiday, their appointment may be brought forward by a few days to allow collection of medication and ensure continuity of treatment. ● This PGD does not allow additional medication to be supplied to enable patients to stock up.
Monitoring and review	● Reassess clinical benefit, tolerability and target weight at each visit. Review is every 4 weeks, at each pen supply. ● If the patient has not lost at least 5% of their initial body weight after 6 months on the maximum tolerated dose, a decision is required on whether to continue treatment, taking into account the benefit-risk profile in that patient. ● When the patient reaches their target weight, discuss whether to continue treatment to maintain it. ● If the patient has used Mounjaro in the past and wants to recommence treatment, the dose must be titrated again following the schedule above. The BMI inclusion criteria for initiation must be reapplied if more than 2 months have passed since discontinuing treatment.
Adverse effects	Very common: nausea, diarrhoea, vomiting, constipation, abdominal pain, decreased appetite and fatigue. Common: hypersensitivity reactions, dyspepsia, eructation, flatulence, abdominal distension, gastro-oesophageal reflux, cholelithiasis, cholecystitis, injection site reactions, hypoglycaemia when used with a sulfonylurea or insulin, dizziness, hypotension-related events, hair loss and tachycardia. Uncommon or rare: acute pancreatitis, delayed gastric emptying, anaphylactic reaction, angioedema, and acute kidney injury usually secondary to dehydration. Counsel on warning symptoms, particularly severe abdominal pain, severe vomiting with dehydration and jaundice. Refer to the current SmPC for full safety information.
Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and

	inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP with whom the individual is registered ● name of healthcare practitioner ● name and brand of medication, and batch number ● date of supply, dose, form, route and quantity supplied ● whether the supply is a start, a restart, a dose escalation, a dose reduction or a continuation, and the previous dose ● the injection site, where the injection is given in the pharmacy ● height, weight and BMI at this visit, and the target weight agreed ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken ● that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. Adults aged 18 and over: keep records for 8 years.

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medicine, together with written lifestyle, diet and physical activity advice and the agreed target weight.
Follow-up advice to be given to patient or carer	Explain the expected pattern of weight loss and that the medicine works alongside diet and activity, not instead of them. Counsel on gastrointestinal side effects and how to manage them, on adequate fluid intake, and on the warning symptoms that need urgent attention: severe abdominal pain, persistent vomiting with dehydration, jaundice, sudden visual loss, or a sustained rise in resting heart rate. Advise the patient that review is every 4 weeks at each pen supply, and that treatment will be reassessed if less than 5% of body weight has been lost after 6 months on the maximum tolerated dose.

Key references

- Summary of Product Characteristics for the product supplied, current version
- NICE guidance on obesity management and on the relevant medicine
- MHRA Drug Safety Updates relevant to GLP-1 receptor agonists
- British National Formulary (BNF)
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates	

Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date
30/9/26	31/7/27

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

To be completed by the adopting pharmacy: the first block is signed by that pharmacy's superintendent or clinical lead, and the second by its professional group signatory. Get Real Health signs the authorship block below, not these.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 002

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development and issue of new PGD	1st May 2026
002	PPH clinical review (J. Wilkins with S. Passmore). Adjusted BMI thresholds for ethnic groups removed from the inclusion criteria as they are not in the SmPC. Gallbladder disease moved from cautions to exclusions as current cholelithiasis or cholecystitis, or cholecystectomy within 3 months. Obesity caused by an endocrinological disorder added as an exclusion. History of suicidal ideation or active severe mental illness moved from exclusions to cautions. Cautions added covering HRT absorption, delayed gastric emptying with INR monitoring on warfarin, pulmonary aspiration under general anaesthesia or deep sedation, and tachycardia. Dosing now states that if more than 2 doses are missed the dose is reduced and re-escalated.	6th August 2026

Version and change record
Version: v011
Issued: 30 September 2026
Supersedes: Version 010, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

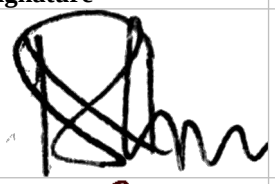
Previous version records
Version: v010
Issued: 14 September 2026
Supersedes: Version 009, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v009
Issued: 11 September 2026
Supersedes: Version 008, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Decision 48: the exclusion for obesity caused by an endocrinological disorder is removed. An endocrine cause of obesity does not exclude; treated hypothyroidism and treated Cushing's syndrome may be supplied. A caution now says that where an endocrine cause is suspected and has not been assessed or treated the GP is informed so that it can be investigated.
Decision 49: SmPC section 6 (storage, shelf life and in-use period) of the Mounjaro KwikPen SmPC, text revised 29 August 2026, is summarised into part 2, and the storage row is aligned to it: after first use the KwikPen may be kept unrefrigerated at not above 30°C for up to 30 days and is then discarded (the row previously said 21 days, which is not the KwikPen figure); the carton and light bullet, which had no SmPC source, is replaced by the section 6.6 inspection and discard instructions; the travel bullet is kept as practical advice.
Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.
Previous version records
Version: v008
Issued: 11 September 2026
Supersedes: Version 007, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Exclusion wording corrected: patients under 18, not adults under 18.
Pregnancy exclusion no longer mentions semaglutide; tirzepatide is stopped at least 1 month before a planned conception (SmPC).

The medullary thyroid carcinoma and MEN 2 exclusion states its basis (US labelling; stricter than the UK SmPC).
The mental health rule that ended the cautions row (do not supply where oversight is absent and concern exists) is now an exclusion criterion, with the caution pointing to it.
The 6 month reassessment is stated one way: 6 months on the maximum tolerated dose, with a recorded decision; the rows said highest tolerated, maximum tolerated and maintenance dose.
A review interval is stated: every 4 weeks at each pen supply.
Adverse effects frequencies corrected to the SmPC: fatigue very common; hypersensitivity, cholelithiasis and cholecystitis common.
Records include the nature of the supply, the previous dose and the injection site where given in the pharmacy.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v007
Issued: 11 September 2026
Supersedes: Version 006, 10 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.
Previous version records
Version: v003
Issued: 9 September 2026
Supersedes: the previous signed version of this document, and any correction notice attached to it.
Change: The concurrent GLP-1 exclusion read "used for weight management". Several GLP-1 medicines are licensed for type 2 diabetes as well, so a patient already taking one for diabetes was not caught and could have been supplied a second. The exclusion now reads "for any indication" and names the products to ask about. This incorporates and withdraws the correction notice of 7 September 2026. No other change.
Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.
Version: v004
Issued: 10 September 2026
Supersedes: Version 003, 9 September 2026
Change: the summary of the governing guidance is added as part 2 of the document, between the cover and the PGD, which is the house format for every Get Real Health PGD. It is summarised from the source it names and adds nothing to it. Nothing else in this document is altered, and this is not a clinical review of the remainder.
Version: v005
Issued: 10 September 2026
Supersedes: Version 004, 10 September 2026
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. No period of validity was stated in any version block.
Quantity: the document said one pen holds one weekly dose and to supply 4 pens a month. The only Mounjaro presentation on the UK market is the KwikPen holding four 0.6 mL doses, as the guidance summary states, so 4 pens was four months' supply. Now one KwikPen (4 doses) per 4-week period.
Version: v006

Issued: 10 September 2026
Supersedes: Version 005, 10 September 2026
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
One rule for insulin secretagogues and insulin. The previous version excluded every sulfonylurea or meglitinide user in one bullet, then excluded them only 'where the GP is unwilling to monitor' two bullets later, then made co-use a referral caution on the next page. The blanket exclusion stands; insulin-treated diabetes is now an exclusion in the same terms; the conditional exclusion and the caution are gone.

Signed on behalf of Get Real Health

Version v011, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.